16.7 Infant of a Diabetic Mother

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • *No specific complaint* **Objective** • RR 72/min • Grunting a little, with some recession • Born at 37 weeks, 4.3 kg	**ACTUAL** **Ineffective breathing pattern** related to delayed lung maturity as evidenced by tachypnea, grunting and retractions.	Within 24 hours, RR 40–60/min and SpO_2 on target.	• Watch RR, SpO_2 and work of breathing. • Give oxygen or CPAP as prescribed. • Keep the baby in a neutral thermal environment. • Report any increase in distress.	Breathing normal: **goal met**.
Subjective • Mother says • "The baby is sleepy and does not feed well." **Objective** • Jittery • Sleepy at feeds • Big for gestational age	**ACTUAL** **Ineffective infant feeding dynamics** related to lethargy and jitteriness as evidenced by poor feeding.	Feeds at least every 3 hours and takes enough.	• Wake the baby for feeds every 2–3 hours. • Help the mother breastfeed; give expressed milk if needed. • Chart intake and output.	Feeding well: **goal met**.
Subjective • *No specific complaint* **Objective** • Mother is diabetic • Glucose 35 mg/dL at 2 hours old	**RISK** **Risk for unstable blood glucose level** as evidenced by risk factors: maternal diabetes and neonatal hyperinsulinism.	Blood glucose stays normal for age.	• Feed within 1 hour of birth and often after that. • Check blood glucose before feeds as the protocol says. • If glucose stays low, give IV dextrose as prescribed. • Watch for jitteriness, poor feeding and seizures.	Glucose normal: **goal met**.